



COLLEGE
OF
MEDICINE

Postmenopausal bleeding

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- Unscheduled bleeding in postmenopausal women is abnormal and may indicate the presence of endometrial cancer.
- Nearly all cases of endometrial cancer (96%) are associated with a thickened endometrium (>4 mm in postmenopausal women), which can be measured by transvaginal ultrasonography.
- If the scan shows a thickened endometrium >4 mm, then histological sampling is essential in the diagnostic evaluation of abnormal bleeding in postmenopausal women.



- Hysteroscopy combined with endometrial biopsy improves the detection of intrauterine pathology and has a high specificity (highly unlikely not to detect cancer).

Causes of vaginal bleeding in postmenopausal women

Polyps	30%
Submucosal fibroids	20%
Endometrial atrophy	30%
Hyperplasia	8–15%
Endometrial carcinoma	8–10%
Ovarian, tubal, cervical malignancy	2%



Endometrial thickness

- The thickness of the endometrial stripe can be measured accurately by transvaginal sonography and it has been estimated that 96% of postmenopausal women with endometrial cancer will have an endometrial thickness (ET) >4 mm.



- Women with PMB whose ET is <4 mm still have a 1–2% risk of having endometrial cancer. TVUS can also show if the endometrial lining is very thin. If so, the bleeding may be due to endometrial atrophy.

It has, therefore, been suggested that as a minimum screening test an endometrial biopsy is required if ET is:

- >4 mm in postmenopausal women
- >16 mm in premenopausal women
- and may be selectively performed in postmenopausal women with ET <4 mm if other historical, clinical or sonographic risk factors are present.

Testing for endometrial cancer

- The preferred investigation of endometrial cancer is biopsy by either blind endometrial biopsy (using Pipelle sampler) or hysteroscopic-guided endometrial biopsy.
- This diagnostic test may be expanded in a 'therapeutic manner' by performing hysteroscopic resection of any identified intrauterine focal lesions (polyps, submucous fibroids), i.e. see-and-treat.